

(W)210mm

(L)297mm

hypersecretory conditions requiring long term treatment (e.g. longer than 3 years), panpriva, as all acid-blocking medicines may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. This diagnosis should be considered in patients with reduced body stores or risk factors for reduced vitamin B12 absorption on long-term therapy or if respective clinical symptoms are observed.

Interference with Laboratory Tests

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient(s) are due to have a test on Chromogranin A level, panpriva treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see section Pharmacodynamic). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of PPI treatment.

INTERACTION WITH OTHER MEDICAMENTS

Medicinal product with pH dependent absorption pharmacokinetics Because of profound and long-lasting inhibition of gastric acid secretion, panpriva may reduce the absorption of drugs with a gastric pH dependent bioavailability, e.g. some azole antifungals as ketoconazole, itraconazole, posaconazole and other medicine as erlotinib.

HIV protease inhibitor

Co-administration of atazanavir and other HIV medications whose absorption is pH-dependent with proton-pump inhibitors might result in a substantial reduction in the bioavailability of these HIV medications and might impact the efficacy of these medicines. Therefore, the co-administration of proton pump inhibitors with atazanavir is not recommended.

If the combination of HIV protease inhibitors with a proton pump inhibitor is judged unavoidable, close clinical monitoring (virus load) is recommended. A panpriva dose of 20 mg per day should not be exceeded. Dosage of the HIV protease inhibitor may need to be adjusted.

Coumarin anticoagulants (phenprocoumon or warfarin)

A few isolated cases of changes in INR have been reported during concomitant administration of phenprocoumon or warfarin. Therefore, in patients being treated with coumarin anticoagulants, monitoring of prothrombin time / INR is recommended after initiation, termination or during irregular use of panpriva.

Methotrexate

Concomitant use of high dose methotrexate (e.g. 300 mg) and proton-pump inhibitors has been reported to increase methotrexate levels in some patients. Therefore, in settings where high-dose methotrexate is used, for example cancer and psoriasis, a temporary withdrawal of panpriva may need to be considered.

Other interactions

Panpriva is extensively metabolized in the liver via the cytochrome P450 enzyme system. The main metabolic pathway is demethylation by CYP2C19 and other metabolic pathways include oxidation by CYP3A4.

No clinically significant interactions with other drugs metabolized with these pathways, like carbamazepine, diazepam, glibenclamide, nifedipine, and an oral contraceptive containing levonorgestrel and ethinyl oestradiol.

An interaction of panpriva with other medicinal products or compounds, which are metabolized using the same enzymes system, cannot be excluded.

Panpriva does not affect the metabolism of active substances metabolized by CYP1A2 (such as caffeine, theophylline), CYP2C9 (such as piroxicam, diclofenac, naproxen), CYP2D6

(such as metoprolol), CYP2E1 (such as ethanol) or does not interfere with p-glycoprotein related absorption of digoxin.

There were also no interactions with concomitantly administered antacids.

No clinically relevant interactions were reported when panpriva was administered concomitantly with the respective antibiotics (clarithromycin, metronidazole, amoxicillin).

Medicinal products that inhibit or induce CYP2C19

Inhibitors of CYP2C19 such as fluvoxamine could increase the systemic exposure of panpriva. A dose reduction may be considered for patients treated long-term with high doses of panpriva, or those with hepatic impairment.

Enzyme inducers affecting CYP2C19 and CYP3A4 such as rifampicin and St John’s wort (Hypericum perforatum) may reduce the plasma concentrations of PPIs that are metabolized through these enzyme systems.

Incompatibilities

This medicinal product should not be mixed with solvents other than those stated in sections Instruction for Use.

PREGNANCY AND LACTATION

Pregnancy

The potential risk for humans is unknown. Panpriva should not be used during pregnancy unless clearly necessary.

Lactation

Excretion of panpriva into human milk has been reported. Therefore, a decision on whether to continue/discontinue breast-feeding or to continue/discontinue therapy with panpriva should be made taking into account the benefit of breast-feeding to the child and the benefit of panpriva therapy to women.

SIDE EFFECTS

The most commonly reported ADRs are injection site thrombophlebitis.

Blood and lymphatic system disorders

Rare: Agranulocytosis

Very rare: Thrombocytopenia; Leukopenia, Pancytopenia

Immune system disorders

Rare: Hypersensitivity (including anaphylactic reactions and anaphylactic shock)

Metabolism and nutrition disorders

Rare: Hyperlipidaemias and lipid increases (triglycerides, cholesterol); Weight changes

Not known: Hyponatraemia, Hypomagnesaemia, Hypocalcaemia in association with hypomagnesaemia; Hypokalaemia; Vitamin B12 deficiency

Psychiatric disorders

Uncommon: Sleep disorders

Rare: Depression (and all aggravations)

Very rare: Disorientation (and all aggravations)

Not known: Hallucination; Confusion (especially in pre-disposed patients, as well as the aggravation of these symptoms in case of pre-existence)

Nervous system disorders

Uncommon: Headache; Dizziness

Rare: Taste disorders

Not known: Paraesthesia

Eye disorders

Rare: Disturbances in vision / blurred vision

Gastrointestinal disorders

Uncommon: Diarrhoea; Nausea / vomiting; Abdominal distension and bloating; Constipation; Dry mouth; Abdominal pain and discomfort

Common: Fundic Gland Polyps (Benign)

Not known: Microscopic colitis

Hepatobiliary disorders

Uncommon: Liver enzymes increased (transaminases, γ-GT)

Rare: Bilirubin increased

Not known: Hepatocellular injury; Jaundice; Hepatocellular failure

Skin and subcutaneous tissue disorders

Uncommon: Rash/exanthema/eruption; Pruritus

Rare: Urticaria; Angioedema

Not known: Stevens-Johnson syndrome; Lyell syndrome;

Erythema

multiforme; Photosensitivity; Subacute cutaneous lupus erythematosus

Musculoskeletal and connective tissue disorders

Uncommon: Fracture of the hip, wrist or spine

Rare: Arthralgia; Myalgia

Not known: Muscle spasm as a consequence of electrolyte disturbances

Renal and urinary disorders

Not known: Interstitial nephritis (with possible progression to renal failure)

Reproductive system and breast disorders

Rare: Gynaecomastia

General disorders and administration site conditions

Common: Injection site thrombophlebitis

Uncommon: Asthenia, fatigue and malaise

Rare: Body temperature increased; Oedema peripheral.

Infection/ infestations

Clostridium difficile associated diarrhoea

SYMPTOM & TREATMENT OF OVERDOSE

There are no known symptoms of over dosage in man.

Systematic exposure with up to 240 mg administered intravenously over 2 minutes were well tolerated. As panpriva is excessively protein bound, it is not readily dialysable.

In the case of overdose with clinical signs of intoxication, apart from symptomatic and supportive treatment, no specific therapeutic recommendations can be made.

Effects on ability to drive and use of machines

Panpriva is not expected to adversely affect the ability to drive or use machines. Adverse drug reactions such as dizziness and visual disturbances may occur. If affected, patients should not drive or operate machines.

Instructions for Use

Instructions and use/handling

A ready-to-use solution is prepared by injecting 10 ml of sodium chloride solution into the vial containing the powder. This solution may be administered directly or may be administered after mixing with 100 ml sodium chloride infusion solution or 5% Dextrose intravenous infusion.

Panpriva should not be prepared or mixed with solvents other than stated.

After preparation the solution must be used within 12 hours.

From a microbiological point of view the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user and would normally not be longer than 12 hours at not more than 30°C after reconstitution and dilution.

The drug should be administered intravenously over 2-15 minutes.

The contents of the vial are for single use only. Any product that has remained in the container or the visual appearance of which has changed (e.g. if cloudiness or precipitation is observed) should be disposed of in accordance with local requirements.

STORAGE CONDITION

Store below 30°C. Protect from light. Do not freeze Retain in carton until time of use.

The solution will remain stable for 12 hours at room temperature (30°C) after reconstitution and dilution. From a microbiological point of view the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

Infusion solution	Period of stability (hours)
0.9% Sodium Chloride 5% Dextrose	12 hours

SHELF LIFE

2 years

The solution will remain stable for 12 hours at room temperature (30°C) after reconstitution and dilution. From a microbiological point of view the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

Infusion solution	Period of stability (hours)
0.9% Sodium Chloride 5% Dextrose	12 hours

DOSAGE FORMS AND PACKAGING AVAILABLE

- 10 x 10 mL vial (10R clear glass vial) of 40 mg Powder for Solution for Injection. 10 vials per unit PVC tray were packed in paper carton.
- 1 x 10 mL (10R clear glass vial) of 40mg Powder for Solution for Injection. 1 vial packed in paper carton.

PRODUCT REGISTRATION HOLDER

/ MANUFACTURER

Pharmaniaga Lifescience Sdn Bhd

(198201002939)

Lot 7, Jalan PPU 3,

Taman Perindustrian Puchong Utama,

47100 Puchong, Selangor Darul Ehsan, Malaysia

2002094-V

Revision Date: 12-October-2021

attn		customer Pharmaniaga LifeScience Sdn Bhd				date 15.10.2021	
4 colours	process colours:	Cyan	Magenta	Yellow	Black	Please Chop & Sign For Approval	
size	(L) 297 x (W)210mm			material	60g Simili		
description	Panpriva Insert(2002094-V)–Back			finishing			
 FocusPrint SDN BHD		t: 603-8766 6030 f: 603-8766 6033 (Factory) website: www.focusprint.com.my					
artwork prepared by: cynthia yap		email: graphic@focusprint.info (Graphic Dept)				date:	

IMPORTANT ! Please note that this colour print is for visual purpose only. Output colour might differ in actual production.

ARTWORK LOG		
Version no.	Date	Reason for Change
01	11.11.2019	- Amend wording.
02	18.02.2021	- New artwork received
03	04.03.2021	- Amend wording at back page.
04	13.10.2021	- Amend wording and revision date.
05	15.10.2021	- Remove wording range.